
The Four Numbers That Predict Your Heart Attack

BEFORE YOUR CHOLESTEROL DOES

The blood tests your panel misses.
Built for South Asian men.



A LETTER FROM

Vaidya Aran

Cardiometabolic Physician

I am not writing this from a distance.

I lost a close friend at 34. He looked fine. Stayed active. His cholesterol came back normal three months before he died. His doctor saw nothing to worry about.

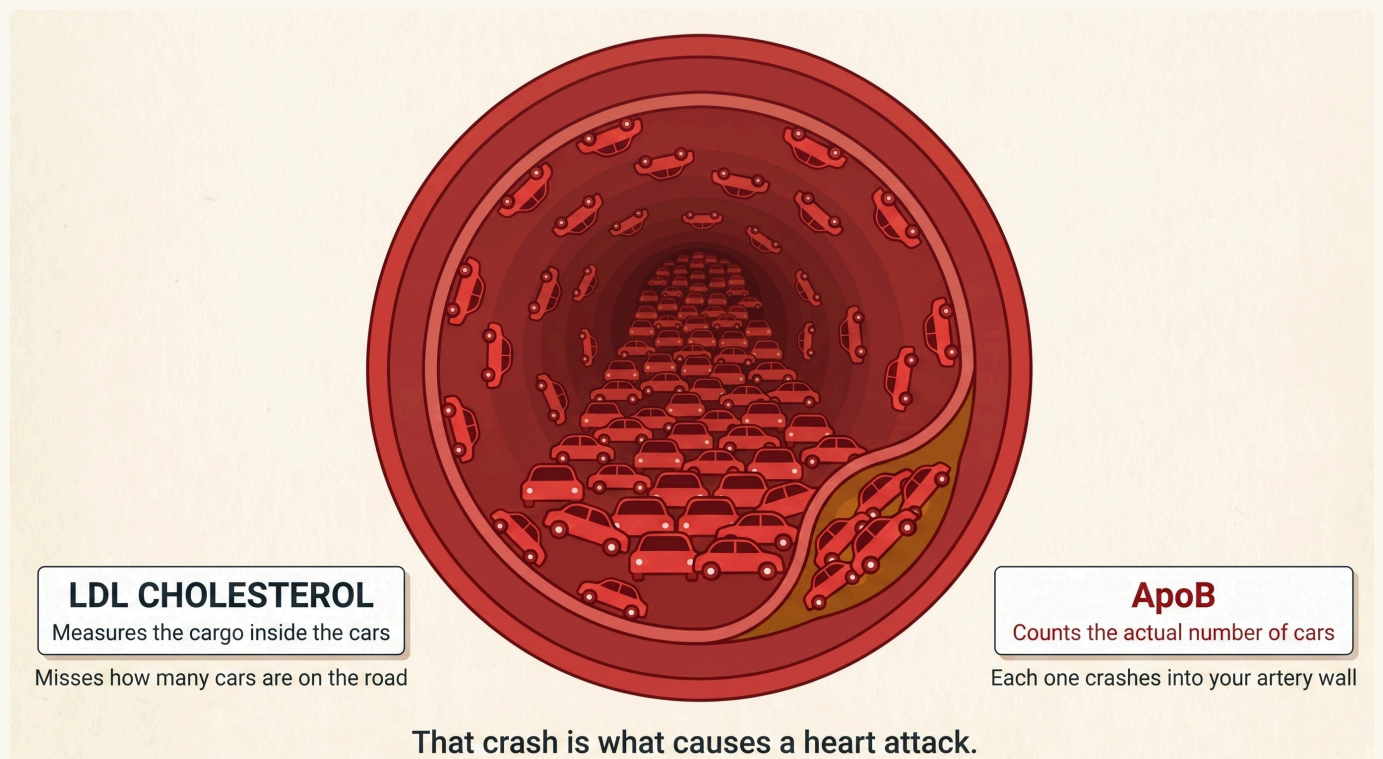
He was not unlucky. He was under-tested.

The blood test his doctor ran, the one most of us get every year, was never designed to catch what kills South Asian men early. It measures the wrong thing. There are four numbers that see the real risk. Numbers that predict a heart attack years, sometimes decades, before it happens.

The tests exist. Nobody ordered them for you.

This guide changes that.

"My friend's doctor measured the cargo. He missed the cars."



The Four Numbers

Your last blood test probably missed all of them.

Screenshot this page. Bring it to your next appointment.

ApoB

< 70 mg/dL

The cars. Not the cargo.

Each ApoB particle is a tiny carrier that can embed into your artery wall, build up over time, and block blood flow to your heart. That blockage is a heart attack. Standard cholesterol counts the cargo weight. ApoB counts the carriers themselves.

There is no safe number. The lower your ApoB, the lower your risk, full stop. The reference range on your blood test is not your target. Ask what your optimal number is, not just whether you are in range.

Ask for: "Can you add ApoB to my blood panel?"

If high: Effective and safe treatments exist. Ask your doctor what is right for you.

Lp(a)

< 75 nmol/L

The card you were born with.

Lp(a) is genetic. You inherited it. Diet, exercise, and most medications will not change it. If it is elevated, it increases your ApoB particle burden directly and makes every other risk factor in this guide hit significantly harder.

You only need to test this once in your life. But you need to know the number.

Ask for: "Can I get my Lp(a) tested? It is a once-in-a-lifetime test."

If high: Treatments are in clinical trials. Ask your doctor about the latest options. Managing your other three numbers aggressively becomes critical.

hs-CRP

< 0.6 mg/L

How inflamed your arteries are right now.

Inflammation makes artery damage worse. It makes vessel walls more porous, more likely to trap particles, and more likely to rupture. High hs-CRP means this process is already active.

The biggest driver in South Asian men is visceral fat, the fat packed deep around your organs, around your liver and stomach. This is not the fat you can see or pinch on the outside. You can carry dangerous amounts of it at a completely normal body weight.

Ask for: "Can you add high-sensitivity CRP to my panel?"

If high: Visceral fat is the priority target. Safe and effective fat loss treatments now exist alongside diet and exercise. Ask your doctor.

HOMA-IR

< 2.0

How hard your pancreas is working.

Insulin is a key. Your cells are like tiny rooms, each with a door. When you eat, insulin turns the key, the door opens, and sugar enters your cells to become energy.

Now imagine the locks get rusty. Visceral fat, packed around your liver and pancreas, does exactly this. Your body cannot tell a rusty lock from a missing key. So it makes more keys. More and more insulin, just to force the doors open. Blood sugar looks normal, but only because your pancreas is working five times harder than it should.

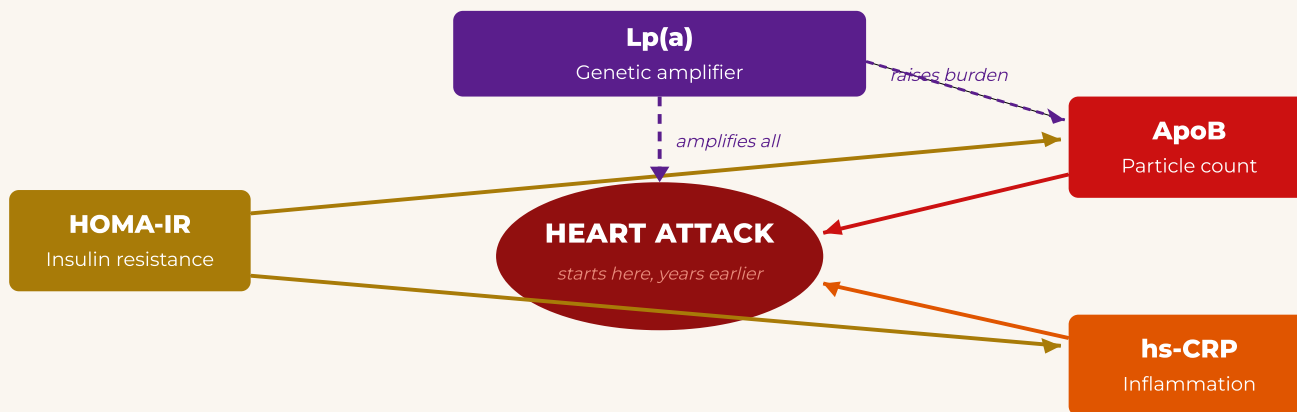
HOMA-IR catches this years before blood sugar moves. For South Asian men, the locks rust earlier, even at a normal weight, even with a normal BMI.

Ask for: "Can you add fasting insulin to my panel so I can calculate HOMA-IR?"

If high: Visceral fat is the root cause. A fibre and protein rich diet helps, specific targets vary. Safe and effective fat loss treatments now exist alongside exercise. Ask your doctor.

These are not four separate problems. They are one system.

Insulin resistance drives visceral fat. Visceral fat produces more ApoB particles and raises inflammation. More particles means more material embedding into artery walls. Higher inflammation makes those walls more vulnerable. Elevated Lp(a) increases ApoB burden directly and amplifies every part of this process. This is why a normal cholesterol result can coexist with a heart attack ten years later.



WHAT TO SAY TO YOUR DOCTOR — PRINT THIS PAGE

"I would like to add four specific tests to my next blood panel. I have been reading about heart attack risk in South Asian men and I want to establish my baseline on each of them."

- **ApoB** — fasting blood draw, report in mg/dL. Target: below 70.
- **Lp(a)** — once-in-a-lifetime. Confirm the unit is nmol/L.
- **hs-CRP** — specify high-sensitivity CRP, not standard CRP. Target: below 0.6.
- **Fasting insulin** — combined with fasting glucose, this gives your HOMA-IR score.

If your doctor will not order these: all four are available privately at any accredited diagnostic lab without a referral. ApoB and hs-CRP typically cost less than a meal out.

You now know more than most South Asian men ever will.

SEND THIS TO SOMEONE YOU ARE WORRIED ABOUT

Your brother. Your cousin. The friend who keeps saying his cholesterol is fine. He almost certainly has not heard of ApoB. He has not had his Lp(a) tested. This guide takes ten minutes to read. It may be the most useful ten minutes he spends on his health this year.

Forward the email. Share the link. Screenshot page three and send it. Whatever gets it to him.

FOLLOW VAIDYA ARAN FOR THE WEEKLY BREAKDOWN

Every week, Vaidya breaks down exactly what is happening inside South Asian men's bodies and what to do about it. No generic Western advice. No cholesterol myths.

Vaidya Aran **@thebrownheart** on TikTok

Vaidya Aran **@thebrownheart** on Instagram

A QUESTION ABOUT YOUR RESULTS?

Reply to the email that brought you this guide. Vaidya reads every one.

Brown Heart is an AI-assisted health education brand. Vaidya Aran is an AI-generated physician persona. Clinical content is authored and reviewed by a DHA-licensed physician with postgraduate training in biomedical informatics. This guide is for information only and is not a substitute for personal medical advice. Always discuss results and treatment decisions with your own doctor. Key references: Sniderman et al., JAMA Cardiology 2019; Tsimikas, JACC 2017; Ridker PM, Circulation Research 2016; Matthews et al., Diabetologia 1985; Enas et al., Indian Heart Journal 1996. · thebrownheart.health